MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 17 OF 28

MFT 717

Psychopathology in Families

15 teaching weeks · 1 practical · 1 worked case

SEMESTER THREE

1.71 units · Prerequisite: MFT 707, 715

> “Severe mental distress is not caused by families, and it is not treated without them.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 717 — Psychopathology in Families

MSc Marriage and Family Therapy · Semester Three · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 707, 715

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	**Psychopathology in Families**	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Severe mental distress is not caused by families, and it is not treated without them. This course builds the knowledge and the nerve to work with psychosis, mood disorders, severe anxiety, personality-level difficulties and eating disorders in a relational frame — alongside medication and psychiatry, not instead of them.

How to use this book

The course deliberately reverses a historical mistake. For decades, family therapy implied that families caused severe mental illness; that legacy still harms families who arrive expecting to be blamed. Each week therefore begins with what is known about aetiology, then asks what a family therapist actually contributes.

1. Course description

The course covers the major categories of severe psychopathology as they present in family life: psychotic disorders, bipolar disorder, major depression, severe anxiety and OCD, personality-level difficulties, eating disorders, and the interface with substance use. It emphasises **family psychoeducation**, **expressed emotion**, **family accommodation**, **relapse prevention**, **caregiver burden**, the **recovery model**, and respectful collaboration with psychiatric services.

2. Learning outcomes

#	Outcome	Assessed
LO1	Describe the aetiology and family impact of major disorders	Exam
LO2	Deliver family psychoeducation for psychosis or bipolar disorder	Skills test
LO3	Assess and reduce expressed emotion and family accommodation	Case study
LO4	Work with caregiver burden and family adaptation	Case study
LO5	Collaborate with psychiatric services and understand medication	Viva
LO6	Avoid both blaming and neglecting the family	Reflective portfolio
LO7	Apply a recovery-oriented frame	Position paper

3. Assessment

Component	Weight	Due
Family psychoeducation session observed (40 min)	30%	Week 8
Case study: family intervention for severe mental illness (2,500 words)	35%	Week 12
Examination	25%	Week 16

Component	Weight	Due
Reflective portfolio	10%	Ongoing

4. The fifteen-week plan

Week	Topic	Practical
1	Historical legacy: from schizophrenogenic mothers to family partnership	Timeline and critique
2	Psychosis I: what it is and what families see	Symptom and experience mapping
3	Psychosis II: family intervention and expressed emotion	EE assessment
4	Family psychoeducation in practice	Session delivery
5	Bipolar disorder and the family	Relapse signature mapping
6	Depression in relational context	Couple-based depression work
7	Severe anxiety, OCD and family accommodation	Accommodation reduction plan
8	**Psychoeducation observed**	Assessed
9	Personality-level difficulties and the family	Boundary and validation work
10	Eating disorders and the family	FBT principles
11	Caregiver burden, siblings and the household	Carer support plan
12	**Case study due**; medication, psychiatry and collaboration	Medication interface
13	Risk, hospitalisation and the family	Admission and discharge planning
14	Recovery, stigma and the family's future	Recovery plan
15	Integration: partnership without blame	Case presentations

Week 1 — The historical legacy

The mistake. Mid-twentieth-century theories blamed mothers for schizophrenia ("schizophrenogenic mother"), attributed psychosis to family communication ("double bind"), and described "families of origin" as pathogenic. These ideas were poorly evidenced, deeply damaging, and are still remembered by families.

The correction. Contemporary research shows that families are neither the cause nor the cure of psychotic disorders, but they are among the strongest predictors of outcome — through support, monitoring, accommodation and emotional climate.

What family therapy offers now:

- psychoeducation (reliable information, in plain language)
- communication and problem-solving training

- reduction of criticism, hostility and over-involvement where they are high
- relapse prevention and early-warning planning
- support for carers
- help with the ordinary relational life that the illness interrupts

> **PRACTICAL**
> Read one historical text and one contemporary family-intervention trial. Write 400 words on what changed, and what harm the earlier framing did.

Week 2 — Psychosis

What it involves. Hallucinations, delusions, disorganised thinking, negative symptoms (flat affect, anhedonia, withdrawal), and cognitive impairment. Onset is typically late adolescence or early adulthood, which means it lands in the middle of a family's launching stage.

What families experience. Fear, grief, confusion, exhaustion, stigma, violence in a minority of cases, and a chronic state of vigilance. Siblings often report invisibility; parents often report marital strain.

What helps (evidence summary). Antipsychotic medication is the foundation. Family intervention (typically 10–12 sessions of psychoeducation plus communication and problem-solving) reduces relapse and improves functioning. Supported employment, cognitive remediation and long-term follow-up matter.

What to avoid. Blaming; interpreting delusions symbolically; colluding with the illness ("he cannot help it, so nothing is required of him"); and treating the family as the patient.

Week 3 — Expressed emotion and family intervention

Expressed emotion (EE) comprises criticism, hostility and emotional over-involvement. High EE predicts relapse in schizophrenia and other conditions, though the effect is smaller than early research suggested and is bidirectional (illness severity also raises EE).

Assessing EE without blaming. Ask about the last month: how many arguments, what triggered them, how the carer responded, how much time is spent managing symptoms, and whether the carer feels able to have a life of their own.

The compassionate reframe. High criticism is usually exhaustion; over-involvement is usually love and fear. Naming that — before teaching any skill — is what makes the intervention acceptable.

Script: "You have been managing something terrifying for four years. Of course you are exhausted and sometimes sharp. The aim of these sessions is not to make you nicer; it is to give you back some room."

Communication training then teaches: making positive requests, active listening, expressing negative feelings safely, and problem-solving in structured steps.

Week 4 — Family psychoeducation

The format. 6–12 sessions, ideally including the person with the diagnosis and their carers, with a combination of information, communication training and problem-solving.

A single-session structure.

Segment	Content
1. What the family already knows	Elicit their model of the illness
2. What is known	Symptoms, course, causes (multi-factorial), treatment, prognosis
3. What helps	Medication adherence, sleep, stress reduction, early warning signs, routines
4. What the family can do	Communication, problem-solving, reducing accommodation, protecting their own life
5. What to do in a crisis	Numbers, triggers, what to say, who to call
6. Questions	Ten minutes of honest answers

Language. Plain, non-technical, and hopeful without being false: "This is a serious condition that can be managed. It is not anyone's fault, and it is not a life sentence."

Week 5 — Bipolar disorder

What families see. Periods of elevated mood, reduced sleep, grandiosity, irritability, risk-taking, and then depression. The family's experience is often of a person they recognise only intermittently.

Family tasks. Recognising the **relapse signature** (early signs of an episode in this person), agreeing a plan in advance, protecting sleep, managing money during highs, and responding to risk.

The relapse signature plan.

1. List the person's own early signs (e.g. sleeping 4 hours and feeling fine; starting three projects).
2. Agree what the family will do (e.g. call the psychiatrist; take over the bank card; reduce stimulation).
3. Agree what the family will not do (argue about whether an episode is real; wait for a crisis).
4. Review after each episode.

Psychoeducation plus family-focused treatment has good evidence for relapse prevention in bipolar disorder.

Week 6 — Depression in relational context

Two directions. Depression is influenced by relationships (criticism, conflict, isolation, caregiving burden), and it influences them (withdrawal, irritability, reduced libido, helplessness).

Couple-based interventions have good evidence for depression, particularly where relationship distress is present. Key mechanisms: reducing criticism and hostility, increasing positive exchange, addressing attributional blame, and supporting behavioural activation with a partner's help.

What not to do. Treat the depressed person as the sole problem; ask the partner to become a therapist; ignore the couple's sexual and practical reality; or reassure prematurely.

> **CASE**
> A husband's depression follows a job loss. The couple's cycle: he withdraws, she takes over the household, he feels useless, she feels abandoned. Intervention: behavioural activation with agreed tasks that restore his role, plus work on her anxiety about relinquishing control.

Week 7 — Severe anxiety, OCD and accommodation

Family accommodation is the family's participation in the anxiety: answering the same question forty times, checking locks, avoiding supermarkets, providing reassurance, taking over tasks.

Why it persists. It reduces distress immediately, which reinforces it for both parties.

Reducing accommodation without cruelty. Build a hierarchy with the family; reduce one item at a time; agree what the person will do instead; expect a temporary increase in distress; and rehearse the script: "I love you, and I am not going to answer that again. I will sit with you for ten minutes instead."

For OCD specifically: psychoeducation about the function of compulsions; family agreement not to participate in rituals; ERP as the treatment of choice, with the family as allies.

Week 8 — Observed psychoeducation session

Forty minutes: eliciting the family's model, delivering information in plain language, teaching one communication skill, agreeing a crisis plan, and answering questions honestly.

Week 9 — Personality-level difficulties

Caution with language. "Personality disorder" is a contested and stigmatising label. Many clinicians describe "personality-level difficulties" or "complex relational patterns" and focus on the pattern rather than the category.

What families experience. Emotional storms, self-harm, idealisation and devaluation, fear of abandonment, and often years of contact with services that have not helped.

What helps.

1. **Validation before change:** "Given what you have survived, this reaction makes sense."
2. **Consistency:** the same rules, the same times, the same responses.
3. **Boundary clarity:** "I will call you back at six" — and doing it.
4. **Family psychoeducation** about emotion regulation and invalidation.
5. **Coordination between services**, because splitting between professionals is part of the pattern.

What harms. Moralising; interpreting the behaviour as manipulation; inconsistency; and treating every crisis as a new emergency rather than part of a known pattern.

Week 10 — Eating disorders

The relational reality. Eating disorders reorganise the household: meals become battlegrounds, siblings become collateral, and parents oscillate between control and helplessness.

Family-based treatment (FBT) for adolescent anorexia is first-line. The stance is **agnostic about cause** and **externalising of the illness**: the family defeats anorexia, and no one is blamed for it.

Phase 1: parents take charge of refeeding, with support, and are absolved of guilt. **Phase 2:** control is returned to the adolescent as developmentally appropriate. **Phase 3:** identity and adolescent issues.

Medical safety is primary. Eating disorders have the highest mortality of any mental disorder. Weight, electrolytes and cardiac status are medical matters; do not work alone.

For adults with longstanding eating disorders, the work is often individual and relational: shame, identity, control, and the couple's accommodation.

Week 11 — Caregiver burden and the household

Assess the carers, not only the patient. Burden is measurable and treatable: use a brief carer measure and ask directly about sleep, work, finances, social life, and their own health.

Siblings. Often invisible, often parentified, often resentful and guilty about the resentment. Give them a session, language for their experience, and permission to have their own life.

The household system. Who is doing the caring? Who has given up what? Is there a "designated carer" whose own life has been suspended? Naming this is often the most useful intervention of the month.

Carer support plan. Respite, information, peer support groups, practical help, and their own GP.

Week 12 — Medication and collaboration

What you need to know (not prescribe).

Class	Common uses	Common side effects to ask about
Antipsychotics	Psychosis, bipolar	Weight gain, sedation, tremor, sexual dysfunction
Antidepressants (SSRIs)	Depression, anxiety, OCD	Nausea, sexual dysfunction, initial agitation
Mood stabilisers (lithium, valproate)	Bipolar	Thirst, tremor, weight, thyroid and renal monitoring
Benzodiazepines	Short-term anxiety	Dependence, sedation, falls

Your role. Support adherence without policing it; explore ambivalence about medication as you would any other ambivalence; report side effects; never advise stopping or changing a dose; and know the prescriber's name.

Collaboration. Write a short letter; attend a case conference; use SBAR on the phone; agree who is responsible for what. Families are often caught between two professionals who have never spoken.

Week 13 — Risk, hospitalisation and the family

Admission is a family event: relief, guilt, fear, and often secrecy. Prepare the family, explain the process, agree visiting, and plan from the first day for discharge.

Discharge is the highest-risk period. Plan: medication supply, follow-up appointment, who is at home, what the early warning signs are, and what to do at night.

When the family is afraid. Take the fear seriously; it is usually based on experience. Build a written crisis plan that the whole household knows.

Week 14 — Recovery, stigma and the future

The recovery model emphasises hope, identity, meaning and self-determination, not only symptom reduction. For families, this means asking: what does a good life look like for this person, and what would support it?

Stigma operates inside the family too: shame, secrecy, "we do not tell anyone", blame of the mother. Addressing internalised stigma is a clinical task.

Planning the future. Housing, work, relationships, and what happens when the parents can no longer care. This conversation is difficult and usually deferred; the therapist's job is to make it possible.

Week 15 — Integration

Four principles.

1. **No blame.** Families are partners, not pathogens.
2. **Information is an intervention.** Accurate, plain-language psychoeducation changes outcomes.
3. **Emotion is contagious.** Reducing criticism and over-involvement helps — and it is achieved by supporting carers, not by instructing them.
4. **Collaboration beats isolation.** Medication, psychiatry, community services and family work together.

Glossary

Accommodation — family behaviours that participate in and maintain symptoms. **Expressed emotion (EE)** — criticism, hostility and emotional over-involvement in carers. **Family-focused treatment (FFT)** — a manualised programme of psychoeducation, communication and problem-solving, with good evidence in bipolar disorder. **Family psychoeducation** — structured information and skills for families living with a disorder. **Prodrome** — the early signs preceding a full episode. **Recovery model** — an orientation emphasising hope, identity, meaning and self-determination. **Relapse signature** — an individual's characteristic early warning signs of an episode.

Self-assessment

Multiple choice

1. Contemporary research on families and psychosis shows that families: (a) cause schizophrenia (b) can significantly influence outcome (c) are irrelevant (d) should be excluded

2. High expressed emotion comprises: (a) criticism, hostility, over-involvement (b) warmth, humour (c) avoidance (d) grief

3. Family accommodation in OCD: (a) reduces long-term symptoms (b) maintains symptoms despite short-term relief (c) is irrelevant (d) should be punished

4. FBT for adolescent anorexia is: (a) blame-based (b) agnostic about cause and externalises the illness (c) individual therapy (d) medication-only

5. A relapse signature is: (a) a diagnosis (b) an individual's early warning signs (c) a blood test (d) a medication

6. The highest-risk period after psychiatric admission is: (a) admission (b) discharge (c) the first year (d) none

7. The therapist's role regarding medication is to: (a) prescribe (b) adjust doses (c) support adherence and report side effects (d) advise stopping

8. Siblings of a person with severe mental illness commonly report: (a) no impact (b) invisibility and guilt about resentment (c) greater support (d) reduced burden

9. The recovery model emphasises: (a) symptom removal only (b) hope, identity, meaning, self-determination (c) institutional care (d) family blame

10. The most damaging historical legacy for families was: (a) too much medication (b) theories blaming families for causing psychosis (c) short admissions (d) family therapy

Answer key

1 b · 2 a · 3 b · 4 b · 5 b · 6 b · 7 c · 8 b · 9 b · 10 b

Short answer

1. State the evidence for family intervention in psychosis in four sentences.
2. What is expressed emotion, and how do you assess it compassionately?
3. Outline a single-session family psychoeducation structure.
4. Give three ways to reduce family accommodation without cruelty.
5. What should a discharge plan contain?

Essay (choose one, 2,000 words)

1. "Families are partners, not pathogens." Trace the history of this claim and its clinical implications.
2. Design a family intervention for a young person with first-episode psychosis, including the first four sessions.
3. Critically discuss the ethics of using the diagnosis "personality disorder" with a family in the room.

Further reading

- Leff, J., & Vaughn, C. (1985). Expressed Emotion in Families. Guilford.

- Kuipers, E., Leff, J., & Lam, D. (2002). Family Work for Schizophrenia (2nd ed.). Gaskell.
- Miklowitz, D. J. (2008). Bipolar Disorder: A Family-Focused Treatment Approach (2nd ed.). Guilford.
- Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa (2nd ed.). Guilford.
- National Institute for Health and Care Excellence. (2014/2020). Psychosis and Schizophrenia in Adults (CG178).

Next: MFT 718 — Intimacy and Relationship Dynamics